National Cancer Control Programme (NCCP) 00692 · clinician draft

Weekly cetuximab + FOLFOX-4

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · chemotherapy and antibody

Active medicines: Cetuximab + Oxaliplatin + Folinic acid + Fluorouracil

Emetogenic risk: Moderate

Medicine	Dose / route	When
Chlorphenamine	10 mg intravenous (IV) bolus	At least one hour before cetuximab Repeat for subsequent cetuximab infusions; assess prior infusion reactions.
Dexamethasone	8 mg IV bolus	At least one hour before cetuximab on Day 1 This corticosteroid also covers the treatment-day corticosteroid part of antiemesis where appropriate. Pharmacy must reconcile route, dose and timing; do not automatically add a second 8 mg antiemetic dose.
Ondansetron	16 mg PO once	Before Day 1 chemotherapy NCIS V8 moderate-risk row; consider QT risk, low electrolytes, interactions and the individual prescription.
Dexamethasone	8 mg PO daily	Days 2 and 3 after the administered Day 1 chemotherapy NCIS V8 delayed prophylaxis. Reconcile with any other steroid prescription; re-anchor dates if treatment is delayed.
Metoclopramide	10 mg PO up to three times daily as required	Rescue nausea treatment only as prescribed NCIS V8 rescue option; check contraindications, renal function, interactions and prescribed duration.

Additional phase-specific support

- Antiemetic classification refers to the active treatment day. Reconcile duplicate steroid entries before prescribing.
- No automatic granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) schedule is added; use the patient's neutropenia history, risk and consultant plan.
- Cetuximab premedication and infusion observation remain necessary even on an antibody-only day.
- Start local EGFR skin care; monitor magnesium, calcium and potassium before and periodically during therapy, then monthly for two months after the final dose.
- Cold exposure can precipitate oxaliplatin neurotoxicity; do not recommend ice chips during fluorouracil in this combination. Review neuropathy every two cycles and sooner if symptoms interfere with function.
- Confirm line care, pump contact details and disconnection arrangements; report chest pain, severe early fluorouracil toxicity or pump/line problems urgently.
- NCCP 00692 v2 p1 permits omitting observation after two tolerated cetuximab doses, whereas current EMA Erbitux SmPC sections 4.2/4.4 require monitoring during and for at least one hour after every infusion. Reconcile this conflict with oncology pharmacy/local policy; do not automatically waive monitoring. Chemotherapy must start no earlier than one hour after cetuximab ends.
- FOLFOX4 uses two separate 22-hour fluorouracil infusions on Days 1 and 2, with a second treatment visit; never substitute a single 46-hour modified-FOLFOX6 pump schedule.
- Cetuximab and FOLFOX may be delayed independently. Reconcile actual treatment days, component-specific recovery and previous dose changes before updating support prescriptions.
- Intolerable cetuximab skin reactions require interruption even when graded below 3 under the EMA product information; review infection and supportive skin care promptly.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00692 v2 · p6 · NCCP NCIS antiemetics V8 (2025), Table 1 and multi-day principles

Sources checked 2026-11-05 · SACTCheck v0.84.36 · Regimen-level drafts; prescriber and oncology-pharmacy verification required